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AMENDMENT PAGE

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KEMENTERIAN KESIHATAN MALAYSIA

OPERATIONAL POLICY

PSYCHIATRY AND MENTAL HEALTH
DEPARTMENT

HOSPITAL SERIAN

2022

Second edition

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Visi, Misi, Objektif

* akan disemak semula setiap 3 tahun atau jika terdapat sebarang perubahan

1. Kementerian Kesihatan Malaysia (KKM)

Visi

Negara mengembleng tenaga kearah kesihatan yang lebih baik.

Misi

1. Untuk memudahkan dan membolehkan rakyat :
 - i) Mencapai sepenuh potensi mereka dalam kesihatan
 - ii) Menghargai kesihatan sebagai aset paling berharga
 - iii) Mengambil tanggungjawab dan tindakan positif demi kesihatan mereka
2. Untuk memastikan sistem kesihatan berkualiti tinggi iaitu :
 - i) Mengutamakan pelanggan
 - ii) Saksama
 - iii) Tidak membebankan
 - iv) Cekap
 - v) Wajar mengikut teknologi
 - vi) Boleh disesuaikan mengikut persekitaran
 - vii) Inovatif
3. Dengan menekankan:
 - i) Sifat penyayang profesionalisme dan kerja berpasukan
 - ii) Sifat menghormati
 - iii) Penglibatan masyarakat

Objektif

Untuk membantu seseorang Individu untuk mencapai dan mengekalkan satu taraf kesihatan bagi membolehkannya menjalankan kehidupan ekonomi dan sosial yang produktif.

Ini boleh dicapai dengan menyediakan perkhidmatan bercorak penggalakan, pencegahan, rawatan dan pemulihan yang cekap, sesuai dan berkesan dengan memberi penekanan kepada golongan-golongan yang kurang bernasib baik.

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2. Jabatan Kesihatan Negeri Sarawak (JKNS)

Visi

Supaya Sarawak menjadi sebuah negeri yang terdiri daripada individu, keluarga dan masyarakat yang sihat melalui sistem kesihatan yang adil dan saksama, cekap, mampu diperolehi, berteknologi sesuai, berupaya berubah mengikut persekitaran serta serasi dengan pelanggan. Sistem ini akan mengutamakan kualiti, inovasi, peningkatan taraf kesihatan serta menghormati martabat insan dan menggalakkan tanggungjawab individu dan penyertaan masyarakat ke arah peningkatan mutu kehidupan.

Misi

Untuk mewujudkan penglibatan dan penyertaan masyarakat Sarawak untuk kesihatan bagi merangsang dan memudahkan rakyat Sarawak untuk :

- Mencapai sepenuhnya kemampuan kesihatan mereka
- Menghargai kesihatan sebagai aset yang bernilai
- Mengambil langkah positif meningkatkan lagi dan
- Mengekalkan status kesihatan bagi menikmati kehidupan yang lebih bermutu.

Objektif

Membekalkan perkhidmatan kesihatan kepada orang ramai terutama sekali penduduk-penduduk luar bandar untuk pencapaian keadaan sihat dan untuk menggalakkan mereka mengambil bahagian dalam proses penukaran sikap kearah mencapai tahap kesihatan yang baik serta mengelakkan dari berlaku wabak penyakit dan menjauhkan daripada bahaya-bahaya yang boleh menjejaskan kesihatan mereka.

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3. Hospital Serian

Visi

Menjadi sebuah Hospital Berpakar Minor yang terunggul dan berkualiti di dalam Sarawak

Misi

- Mempromosi dan meningkatkan mutu kesihatan
- Mencegah penyakit berdasarkan garis panduan yang ditetapkan oleh KKM
- Memberi rawatan dan pemulihan yang selamat, berkualiti, cekap dan berkesan kepada pesakit, keluarga waris supaya mereka dapat menikmati kehidupan yang lebih bermutu
- Memberi latihan kepada semua anggota untuk meningkatkan kemahiran dan kompetensi

Objektif

- Meningkatkan taraf kesihatan dan memberi kesedaran bahawa kesihatan itu adalah tanggungjawab individu dan masyarakat
- Memberi perkhidmatan kesihatan dari segi pencegahan, diagnostik rawatan perubatan, pemulihan dan pendidikan kesihatan yang berkesan, selamat dan professional.
- Memberi perkhidmatan selaras dengan keseluruhan objektif Kementerian Kesihatan Malaysia.
- Memberi latihan kepada semua anggota untuk meningkatkan kemahiran dan kompetensi.

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4.Jabatan Psikiatri dan Kesihatan Mental, Hospital Serian

Visi

Memberikan perkhidmatan psikiatri dan kesihatan mental yang saksama dan berkualiti kepada masyarakat di Serian kearah melahirkan masyarakat yang sihat dari segi kesihatan mental.

Misi

- Untuk menyediakan khidmat kesihatan mental di kalangan komuniti sejagat
- Untuk meningkatkan pengetahuan kesihatan mental di kalangan komuniti sejagat

Objektif

- Untuk memastikan golongan masyarakat setempat mudah untuk mendapatkan khidmat kesihatan mental
- Untuk meringankan beban pesakit mental yang tinggal jauh dari Bandar Kuching untuk mendapat rawatan terutamanya yang mempunyai masalah logistik
- Untuk meningkatkan pengetahuan kesihatan mental di kalangan Kesihatan Primer dan komuniti

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Objectives

The objectives are in line with the National Mental Health Policy and is to provide:

- Comprehensive, effective and efficient services, including outpatient, inpatient, community outreach services, special services looking into child and adolescents, elderly, co-morbidity with substance use, etc.
- Integrated psychiatric services in mainstream general health care and continuous services from health clinics to a hospital and vice versa.
- Promotion of mental health services for the general population and specific target groups.
- Multi-sectoral collaboration and opportunity for community participation.
- Training for all levels of human resources.
- Monitoring of standards by key performance indicators, NIA, credentialing and privileging special procedures eg electroconvulsive therapy.
- Research (service-oriented, clinical trials, quality assurance) and evidence-based medicine (Clinical Practice Guidelines, etc.).
- Services which comply with legislation (Mental Health Act 2001), respecting human rights of people with mental illness.

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Scopes of services

Scopes of services include:

Hospital with resident psychiatrist

- Promotion of mental health
- Early detection and prompt treatment
- Specialist outpatient care
- Inpatient care
- Hospital-based community psychiatry
- Psychosocial interventions
- Liaison consultation services
- Research
- Training

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Component of services

The range of services to be provided by the psychiatry department in this hospital include:

- Outpatient services
- Inpatient services
- Hospital-based community psychiatry
- Rehabilitation and recovery-oriented services
- Training and research
- Promotion of mental health

Organization

- The Head of Psychiatric services as the national advisor to the MOH on all matters pertaining to the service
- The department shall be headed by a psychiatrist who is appointed under the Mental Health Act 2001.
 - Is responsible for the management of all the components of the service.
 - Collaborates with the National Advisor of Psychiatric Services in formulating strategic plans of service development, policies and procedures.
 - Work closely with the relevant stakeholders such as hospital director, nursing managers and heads of other clinical services in areas pertaining to development, operation and other technical matters.
 - Builds a team of dedicated multidisciplinary staff comprising psychiatrist, trainees, medical officers, staff nurses and assistant medical officers, psychologists, counsellors, occupational therapists, physiotherapists, medical social workers, pharmacist, etc.

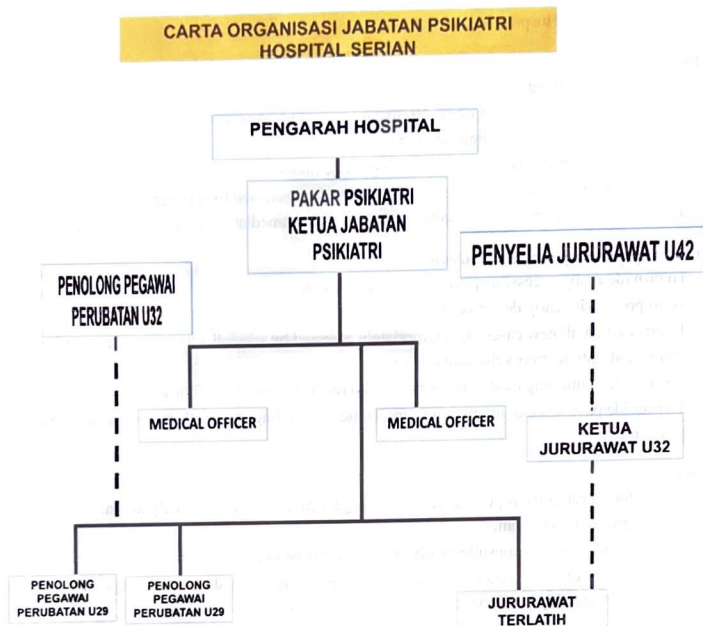
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Organization chart



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Outpatient services

Location

- Outpatient clinic, Hospital Serian

Operations

- Clinic by appointment for
 - Follow up cases: twice a week, 20-30 cases per clinic session
 - New case: 3 cases per clinic session
- Walk in clinic/ emergency: everyday during office hours
- All patients who present with a referral should be first assessed by a triage personnel and later determine the urgency of outpatient evaluation either immediately, early or given appointment.

Objectives of psychiatry outpatient services

- To provide easily accessible psychiatric outpatient service.
- To improve with early detection and early treatment.
- To ensure that all new cases are appropriately assessed by specialist to determine underlying illness and initiate biopsychosocial treatment.
- To provide continuing treatment for those who require longer term follow up.
- To provide psychosocial interventions including counselling, psychotherapies, patient and family education.

Functions

- Triage for all referrals as per agreed format: urgent cases seen immediately and non-urgent cases will be given appointment.
- All new cases must be consulted with specialists at least once.
- Follow up: medical officers can review patients and continue medication. Change of medication should be preferably be discussed with specialist in charge.
- Depot clinics: for stable patients who require continued medication especially those on regular depot injections- given in outpatient clinic
- Defaulter tracing: every patient who has missed one follow up will be contacted as early as possible to arrange for earliest possible outpatient visit.
- Initiating early and prompt outpatient treatment: accurate diagnosis early ensures correct treatment.
- Counselling and psychotherapies when needed: most psychiatric treatment must also include elements of counselling or psychotherapy.

Equipment

- Minimum requirements include office space and furniture.
- Essential equipment includes weighing machine, equipment for vital signs monitoring, tape measure.

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Standards of care

NIA/KPI/HPIA standards should be adhere to. In addition, the following should be noted:

- All patients seen at outpatients should have diagnosis made and be on appropriate treatment.
- Outpatient will need to include monitoring of blood pressure, pulse rate and weight during each clinic visit.
- Management of psychiatric patients is based on the latest best available evidence, Malaysian clinical practice guidelines and guidelines available (refer to folder in psychiatry office computer which include the latest clinical practice guidelines and relevant guidelines).

Operational hours

- Follow up and new case clinic
 - Twice a week (Monday and Thursday), office hours
- Walk in clinic/ emergency: everyday, during office hours
- Counselling and psychotherapies can be done by appointments

Fees

- Follow Akta Fee 1951 and Perintah Fee (Perubatan) 1982

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Inpatient services

Semi Acute Ward

Objectives of ward

- To quickly stabilize the acutely ill psychiatric patients.
- To prevent injury to the patient and staff.
- To form a therapeutic alliance with the patient.
- To form a therapeutic alliance with the caregiver.
- To exclude organic problems for the patient's symptoms.

Admission

- Admission can be from emergency department or clinic.
- Medical officer in emergency department or clinic will screen the case and refer to psychiatric team for decision of admission or outpatient management.
- All new cases must be referred to medical team if indicated to rule out any organic cause
- Any known cases should be rule out of medical emergencies before referral to psychiatric team for further management.
- Psychiatric cases with medical/surgical emergencies should be managed in respective ward with liaison review by psychiatry team
- Psychiatry cases that require admission will be admitted to respective ward/institution as below:
 - Female patient will be admitted to female medical ward
 - Male patient will be admitted to male medical ward
 - Paediatric patient will be admitted to paediatric ward
 - Obstetric patient will be admitted to obstetric ward
 - If any of the above wards are full, the patient will be referred to Hospital Sentosa or psychiatry ward in Hospital Umum Sarawak for admission.
 - Patient with moderate to high risk of suicidal and aggression will be referred to Hospital Sentosa or psychiatry ward in Hospital Umum Sarawak for admission (in view of lack of proper facilities and inadequate staffing to accommodate to strict monitoring required)
- All cases need to be discussed with the psychiatrist.
- Admission of patient under police custody or prisoner needs to be accompanied by the police officers or prison wardens respectively in the ward at all time.

Discharge

- All patients need to be reviewed on the day of discharge and with family members if possible.
- Psycho- education and discharge note will be given to patient.
- Patients may be discharged from ward directly home.

Standard of care

- All patients in this ward shall be reviewed at least once a day by the medical officers, and preferably twice a week by psychiatrist if available.

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- Only the psychiatrist is allowed to make decision to transfer any patients to other psychiatry hospital for further care.
- Relevant investigations shall be ordered and the results traced and reviewed by the medical officer on the same day they are ordered.
- Relatives may be allowed to meet with the medical officer in charge.

Infrastructure and facilities

- A resuscitation trolley, defibrillation machine, oxygen, suction, patient monitoring device which measures blood pressure and pulse shall be present.
- Lockers shall be provided for patients.

Visiting

- Visiting may be restricted by the medical officer or specialist in charge of the patient.

Care plan

- All patients shall have a nursing and a psychiatric care plan.
- The specialist in charge shall determine and review the care plan.
- Discharge planning should be made.

Rehabilitation

- Only in house rehabilitative activities are encouraged.
- Patients may be required to make their own beds and wash up after their meals.
- Psycho education should be started for patients and their relatives.

Restraint

- Type: chemical and physical restraint
- Restraint chart and proper nursing care must be initiated.
- Mental Health Regulation 2010 for the following may be referred
 - Indication for procedure for seclusion and restraint
 - Restrain area
 - Restrain equipment
 - Application of physical means of restraint
 - Indication for chemical means of restraints
 - Procedures for chemical means of restraints

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Management of suicidal patient

- Patients with suicidal risk should be assessed, graded and monitored according to the severity.
- To refer to guideline on suicide risk management in hospitals 2014

General component of care

- Management of psychiatric patients is based on the latest best available evidence, Malaysian clinical practice guidelines and guidelines available (refer to folder in psychiatry office computer which include the latest clinical practice guidelines and relevant guidelines).
- New cases who need admission will require a thorough diagnostic workup.
- Patients who had relapses should be reviewed as to the reason for relapse and appropriate treatment instituted.
- Management of all inpatients should follow the biopsychosocial approach.
- The inpatient services should be a multidiscipline team consisting of psychiatrist, medical officers, nurses, assistant medical officers, pharmacists, counsellors, occupational therapists and health education officers if possible.
- The responsibilities of the inpatient team as mentioned above include:
 - Conduct daily ward rounds
 - Provide inpatient medical procedures (include provide appropriate nursing monitoring regularly (vital signs, pain, sleep and behaviours), prescribing, dispensing and administration of medications, perform investigations with review, and provide counselling, restraint and seclusion as indicated).
 - Arrange for appropriate referrals.
 - Provide for psychiatric care for patients from other disciplines when referral made.
 - Provide a discharge summary and plan.
- Discharge plan should include:
 - Medications to be taken.
 - Type of follow up (outpatient, health centre, community psychiatry unit).
 - Date of follow up
 - Other interventions needed and their dates and location (e.g., place and type for psychoeducation, family intervention, job placement etc).
- When possible, family members should be involved early in management for all patients who are admitted.
- Rehabilitation services that should be incorporated early as inpatient include patient and family education and illness management.
- For all patients admitted, the individual care plan should be initiated.

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Hospital based community psychiatry (HBCP)

- The main aims of this team would be
 - To prevent readmissions
 - To facilitate early discharge
 - To help divert patients from being admitted
- This team must be multidiscipline in nature including psychiatrist, medical officer, staff nurses, or assistant medical officers. Other members of the team may include clinical psychologist, occupational therapists, medical social workers, pharmacist and other supporting staff.
- This service is under the authority of the head of department as stipulated in Mental Health Regulation 2010 (Part III, Section 16).

Components

- Defaulter tracing: some patients default due to logistic reasons. A service to remind them for follow up is important.
- Level 1 and 2 follow up: some patients have more than 1 unmet need and will need regular home visits to ensure medication adherence to maintain their stability.

Follow up of Level 1 and 2 patients

These patients are fairly stable but have tendency to default follow up and medication adherence may be their only need. So, ensuring they stay on medication is key for these patients.

Requirements

- Operational base: Psychiatry office, Hospital Serian
- Service operation: At patient's home as much as possible or nearest venue to home (e.g., nearest clinic etc). Area of operation should be within an acceptable range from the hospital base but not more than 25km or as agreed by HOD.
- Operations: during office hours and normal working days only
- All referrals to HBCP outside of the hospital (polyclinic or klinik kesihatan) should be evaluated at the psychiatric clinic first.
- All internal referral cases requiring community services should complete the referral form with acknowledgement and consent of the next of kin before submitting to the respective team in charge. All cases must be presented directly to the community psychiatry medical officer or specialist in charge.
- Referrals for this follow up can be made from medical officers to medical officers either from outpatients, inpatients or health centres.
- All referrals will be assessed as early as possible, and an agreed care plan made with the HBCP team.

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Objectives of follow up services

- To ensure medication adherence.
- To help patients attain full functioning and improve quality of life.
- To help patients get in touch with other relevant services and agencies.
- To provide relevant psychosocial interventions to patients and families.

Functions

- The team is a multidisciplinary team lead by psychiatrist. Other team members include medical officers, staff nurses and assistant medical officers. Other members of the team may include counsellor, occupational therapist, social worker and support staff.
- All patients referred for follow up will be assessed and discussed by the team.
- A case manager will be appointed to look after the medication needs of a particular patient
- The case manager will have a fixed number of patients on his/her workload
- The type of contact will be determined by the team either at home or nearest to home as possible. Contact may also be at workplace, nearest clinic etc.
- Case manager must work closely with patients' families to ensure medication adherence.
- Case manager may provide other direct services but can also arrange for patients to obtain other services elsewhere.
- Telephone monitoring can also be done to ensure patients adhere to medication visits.

Inclusion and exclusion criteria for follow up services

- Patient's inclusion criteria are:
 - Patient with severe mental illness who has a history of frequent defaulting due to logistic reasons
 - Good psychosocial support
 - Consent from the patient or relatives
- Patient's exclusion criteria are:
 - Personality disorder such as antisocial personality
 - High risk of suicide or aggression
 - Chronic mental disorder without signs of relapse
 - Non engagement to services

Equipment

- Minimum requirements include office space, telephone, furniture and vehicle for transportation.
- Useful equipment includes computer and photocopier.
- Essential equipment includes a bag for medications, disposables, specimen bottles, BP set, tourniquet, stethoscope and documents.

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Standards of care

- Relapse rates are usually low (<0.5%) so all readmissions should be considered a sentinel event and an inquiry made to prevent recurrences.
- There should be regular reviews by medical officers on patient's medication requirements.
- There should be at least one staff (MA/nurse) for every 40 follow up patients.
- All patients on follow up must have a care plan made.
- Management of psychiatric patients is based on the latest best available evidence, Malaysian clinical practice guidelines and guidelines available (refer to folder in psychiatry office computer which include the latest clinical practice guidelines and relevant guidelines).

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Liaison consultation service

Objective

- Provide multidisciplinary specialist assessments and interventions for patients undergoing assessment and treatment in the various disciplines in hospital settings.
- To provide evidence-based treatments for liaison patients.

Operations

- The conditions assessed and treated by psychiatry teams may include:
 - Psychiatric or behavioural presentations in patients with medical conditions
 - Adjustment to physical disease and illness
 - Self-harm and self-injury
 - Co-morbid physical and mental health diagnoses
 - Functional somatic symptoms and syndrome
 - Consent, capacity and mental health act assessment
- Referral system
 - Referrals are made to the psychiatry medical officer on-call and case will be discussed with psychiatrist accordingly.
 - Consent from the patient and family shall be obtained prior to the consultation.
- Psychiatry team shall document the assessment, clinical findings and recommendations to the primary team focusing on the reason for the referral.
- Discharge from liaison psychiatry will be made following completion of treatment or when referral to another service is indicated. Prior to discharge, a review of the care plan will be undertaken and discussed with patient and family members if possible.

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Rehabilitation/ recovery-oriented services

Location

- Can be inpatient or outpatient (outpatient clinic, occupational therapy office)

Operations

- Rehabilitation services should be offered to all patients whether inpatients or outpatients.
- Most evidence-based rehabilitative services are for individuals with severe mental illnesses.
- Rehabilitation services are an integral part of inpatient care.
- Rehabilitation begins with an individualized care plans that addresses the patients' unmet needs and consolidate his/her met needs.
- Rehabilitation must be multimodal in nature.

Objectives of rehabilitation services

- To enable patients to achieve their optimal functioning.
- To enable patients to live independently.
- To empower patients to understand and manage their illness effectively.
- To encourage involvement of families in care.

Functions

- Psychoeducation: both inpatients and outpatients
- Family education: can be done in and outpatients

Interventions

- Formulation of individual care plans
 - As part of biopsychosocial management of all patients, an individual care plan should be formulated. Amongst this plan should include need for patients to attend patient and family education classes and other skill-based intervention available in the facility.
 - A successful recovery service will be multimodal in nature and refers to the concurrent utilization of patient and family education.
- Psychoeducation
 - Psychoeducation includes didactic lectures, giving instructions and involves any provision to patients of information, support and management strategies.
 - This should be an ongoing process and be made available to inpatients as well as outpatients.
 - Essential components include providing information on illness and management, early warning signs detection and crisis intervention and management.

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- Family intervention
 - It includes the following elements: education, communication skills and problem-solving training, stress management, crisis management and identifying early warning signs.
 - It should be made available to all families of patients who had a recent relapse or had been admitted and to families of patients with persisting symptoms.
- Social skills training and/or illness management programs
 - This consists of practicing specific skills such as self-care, conversation, making friends, etc.
 - It is important part of the multimodal approach to recovery.

Operational hours

- Office hours

Fees

- Follow Akta Fee as per government regulations

Inpatient rehabilitation

- Components:
 - Psychoeducation
 - Family intervention
 - Activities of daily living
 - Management of daily routine
 - Grooming and personal hygiene
 - Effective use of leisure time
- Operational policy:
 - All patients admitted to psychiatric hospital should have an individually tailored care plan made for them.
 - The care plan should include details of medications and its intended duration and a multimodal psychosocial intervention.
 - The intervention includes psychoeducation (after recovery from acute symptoms; usually after 3 days) with patients being given some of education before discharge.
 - A family intervention plan should be instituted and patients' and their families should be seen at least once before discharge. Subsequent family intervention strategies include skills training can be done as outpatients.
 - All inpatients should be trained in ADL, grooming, personal hygiene, social skills and effective use of leisure time before discharge.

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Training and research

Training

Ministry of Health Malaysia promotes lifelong learning towards producing excellent and competent human resources. It has introduced the Continuous Professional Development (CPD) to enhance the professional officers in the respective areas of work through participation in activities that contribute to increased knowledge, skills and experience. CPD is an ongoing learning activity conducted for the purpose of enhancing professionalism through increased knowledge, skills and experience. These activities are conducted together with daily activity tasks. Training and development will be a constant and crucial component in developing careers with Ministry of Health. Anonymity on patients and providers involved in any of the training and performance improvement activities should be ensured.

- In service training (Latihan Dalam Perkhidmatan)
 - In service trainings are programs designed to strengthen the competencies of workers while they are on the job.
 - In service training included induction or orientation training, refresher courses, bedside teaching and ward rounds. Posting to various functional areas in psychiatry and mental health services will form an indispensable part of training and development that would help staff to be exposed and be kept abreast of a wide range of health-related issues.
- Continuous Medical Education (CME)
 - CME consists of educational activities that serve to maintain, develop, or increase the knowledge, skills, professional competence and relationships mental health personnel uses to provide services for patients and the public.
 - The content of CME is usually imparted through workshops, journal clubs- critical appraisal, seminars and informal contacts.
- Short Courses and Conference
 - While on-the job training is the backbone of training but opportunities must be provided through formal training in the forms of courses, seminars and conferences offered by various local and overseas training providers for personal and professional development.

Research

One of the main components of the Ministry of Health, Malaysia to promote progress in healthcare and enhance quality of healthcare services is Research. It is desirable, therefore, that research activity is one of the activities of daily work of all officials in the Ministry of Health.

- The department shall provide support and opportunities gearing toward encouraging mental health personnel to carry out research.
- Ensure all local research is registered online through the National Medical Research Register (NMRR). Review research to be done in departments.
- Ensure that clinical research that involves human subjects has Ethics review & approval.
- Ensure that MOH approved research that is eventually published has received prior approval from the National Institute of Health (NIH) and the Director-General of Health, Malaysia.

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Promotion of Mental Health

- Effective mental health promotion, prevention and early intervention strategies will play a key role in improving mental health in the country and reducing the prevalence and impact of mental health problems or illness.
- Approach will be directed at different range of settings including families, schools, workplace and the communities.

Objective

- To promote positive mental health and wellbeing by increasing emotional resilience, reduce vulnerability to mental illness and encourage to seek help when needed.

Topics for mental health promotion

- Mental health for general population
 - General mental health
 - Stress management
 - Anger management
 - Communication skills
 - Parenting skills
 - Assertive training
 - Positive thinking
 - Suicide prevention
 - Relaxation techniques
 - Spiritual intelligence and spiritual health
 - Anxiety
 - Depression
 - Coping with loss and grief
 - Understanding mental illness
 - Removal of stigma to mental illness
- Mental health for children
 - Normal psychosocial development
 - Mental health problems in children
 - Temper tantrums
 - Making friends
 - Sibling rivalries
- Mental health for teenagers
 - Self-awareness
 - Self esteem
 - Self-image
 - Bullying
 - Gender identity

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- Homelessness/ runaways
- Self-injurious behaviours
- Handling separation
- Making friends
- Coping with changes in life
- Mental health for working adults
 - Time management
 - Understanding personalities
 - Life goals
 - Coping with changes in life
- Mental health for elderly
 - Aging gracefully
 - Sleep problems
 - Depression
 - Confusion
 - Elderly abuse
 - Retirement and finance
 - Facing death and dying

Site: All primary health care centers and psychiatry unit/ hospitals

Service operations: all health providers with technical support from mental health workers

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Miscellaneous

1. Medical report writing

- Medical report is a document prepared by a certified medical or dental practitioner/ doctor working with the Ministry of Health of Malaysia {Akta Fi 1951 - Perintah Fi (Perubatan) 1982} with reference to the state of health and treatment received by an individual based on his or her medical records kept in a hospital or any medical or health facility.
- Types of Medical Report
 - Standard medical report (Laporan perubatan biasa) prepared by medical officer.
 - Brief medical report/professional opinion (Laporan ringkas/pendapat) prepared by specialist.
 - Detailed medical report (Laporan perubatan terperinci) prepared by specialist.

Please refer to the medical report writing format document provided by the medical records department entitled, "*Garis panduan penyediaan laporan perubatan di hospital-hospital dan Institusi perubatan - pekeliling ketua pengarah kesihatan bil 16/2010*", for further details.

2. Management of death

- Any death of psychiatry inpatient will need to be reported to police.
- Family members or next of kin of the patient need to be informed as soon as possible.
- Post-mortem needs to be done to identify the cause of death with consent from family member.

3. Management of incident

- All incidents should be reported, investigated, discussed, recorded and action plans implemented.
- All the incident reports shall be compiled.

4. Handover communication

- Medical officer should handover the patients under his or her care to the on call medical officer if required further attention or the medical officer who cover him or her during absence of work i.e., on leave or official duty away from workplace. Similar handover and communication should be done by allied health professionals.

5. Drug prescription, dispensing and administration

- Drug prescription should be done by medical officer/specialist.
- Signature of specialist for prescription with List A items during office hours should be obtained as soon as possible before sending to pharmacy.
- Dispensing and administration of drug should comply with the protocol set.

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6. Blood transfusion

- Blood transfusion shall be done in medical ward if required.

7. Management of cases with an infectious disease

- Notification of notifiable disease according to latest guideline and shall be done within stipulated time line.
- Patients who are started with antibiotics need to be reviewed according to guideline
- Inpatients who required intravenous antibiotic preferably should be managed at respective ward.

8. Internal and external disaster

- All staff shall be aware of contingency plan of internal and external disaster.
- Selected staff will involve in providing psychological first aid to the victims and their family members when instructed.

9. End of life care

- Patient who needs end of life care shall be manage at respective discipline.

10. Staff deployment

- During period of acute shortage of staff (medical officer/allied health professionals), head of psychiatry department shall inform hospital director to discussed on option of internal or external deployment.
 - Internal deployment: to mobilize staff within Hospital Serian to cover for the shortage
 - External deployment: to mobilize staff from hospitals/institutions nearby to cover for the shortage
- Staff with background of psychiatry training or experience is preferred as candidate of deployment.
- Number of staff deployed should be proportional to the number of patients being cared for as in the regulatory requirements and intensity of care provided
- Arrangement of either internal or external deployment will be handled by administrator at hospital level.
- Duty roster of deployed staff should be made available and deployed staff should be preferably being paired with another senior or experienced staff on-duty.

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11. Orientation

- All new staff that join psychiatry department will need to attend structured orientation briefing in accordance with the MSQH Malaysian Hospital Accreditation Standards & Assessment Tool 6th Edition 2021.

12. Safety and performance improvement activities

- Safety and performance improvement activities e.g., mortality & morbidity review, incident reporting and etc should be performed as indicated.
- Results on such activities should be made accessible to staff.
- All documentations of such activities should be kept and confidentiality of psychiatry practitioners, staff and patients should be preserved.

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DASAR OPERASI AM

(STANDARD OPERATING PROCEDURE)

JABATAN PSIKIATRI & KESIHATAN

MENTAL,

HOSPITAL SERIAN

2022

Second edition

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DASAR OPERASI AM JABATAN PSIKIATRI

Rujukan Ke Jabatan Psikiatri

1. Rawatan dan pemeriksaan hanya ke atas kes- kes yang dirujuk .
2. Kes yang dirujuk akan diberi tarikh dan waktu temujanji oleh kakitangan klinik pakar psikiatri.
3. Kes 'Urgent'/ 'Walk In' akan diperiksa pada hari yang sama atau dimasukkan ke dalam wad jika diperlukan .
4. Kes kecemasan akan diperiksa di Jabatan Kecemasan dan akan diberi Tarikh dan waktu temujanji atau dimasukkan ke wad psikiatri jika perlu.
5. Kes yang di rujuk kepada pakar psikiatri adalah untuk tujuan seperti berikut:
 - a) Untuk rawatan selanjutnya.
 - b) Untuk pandangan dan nasihat pakar psikiatri dalam masalah khusus.
 - c) Untuk penyiasatan lanjut yang tidak dapat dijalankan oleh doktor yang memeriksa.
 - d) Seperti yang dinasihat oleh pakar psikiatri untuk pemeriksaan ulangan.

Pesakit yang telah dirawat sebagai pesakit luar atau dalam akan dirujuk semula jika perlu kepada pusat rujukan yang merujuk untuk rawatan susulan. Surat maklumbalas akan diberi bersama-sama maklumat yang perlu dan pelan rawatan juga disertakan supaya pengendalian sewajarnya dapat diberikan kepada pesakit.

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Rujukan Ke Hospital Lain

1. Rujukan ke hospital lain untuk kemudahan rawatan yang tidak terdapat di hospital ini ataupun untuk kemudahan pesakit dari segi kewangan dan pengangkutan di pusat rawatan yang terdekat dengan rumah pesakit (De-centralising) sekiranya pesakit stabil dengan rawatannya.
2. Rujukan kes ke 'Subspeciality Unit' di hospital yang mempunyai pengkhususan tersebut,
3. Rujukan ke hospital pakar lain boleh juga dibuat setelah dipersetujui oleh pakar hospital berkenaan.
4. Rujukan mesti menggunakan borang yang dikhaskan dan mengandungi latarbelakang (clinical history) yang perlu, pemeriksaan fizikal dan rawatan yang telah diberi. Sebab rujukan mestilah ditulis secara jelas.
5. Pesakit yang ingin mendapatkan bekalan ubat dari klinik yang berdekatan dengan rumah akan dirujuk kepada pegawai farmasi yang akan menguruskan proses SPUB.

Dokumentasi Rekod Perubatan Pesakit.

Pakar psikiatri akan memastikan pegawai membuat nota yang tepat, padat dan boleh digunakan.

Maklumat yang berkenaan termasuk :

- Data pengenalan pesakit.
- Latar belakang (clinical history) ,pemeriksaan fizikal dan juga ' Mental State Examination'
- Investigasi/ penyiasatan dan Keputusan.
- Rawatan yang telah diberi.
- Prosidur yang telah dijalankan dan sebab (reason for undertaking procedure).
- Nota susulan dan konsultasi dsb.
- Kemudahan hubungan dengan pesakit , waris , doktor lain, jabatan polis dsb. (Telefon, alamat dsb)

Ringkasan discaj hendaklah disiapkan dalam masa 24 jam selepas pesakit discaj. Laporan perubatan perlu disiapkan dan diserahkan kepada Pejabat Rekod dalam masa 2 minggu dari tarikh permohonan. Untuk kes ketiadaan doktor yang telah merawat pesakit untuk membuat laporan kerana telah bercuti panjang , bertukar dan sebagainya. pegawai perubatan dari Jabatan yang sama akan ditugaskan membuat laporan.

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Latihan

Pakar adalah bertanggungjawab bagi memberi latihan kepada pegawai perubatan dan paramedik/jururawat bagi meningkatkan pengetahuan dan kecekapan agar dapat memberikan perkhidmatan yang terbaik kepada pelanggan di jabatan ini. Di samping itu latihan ini juga bertujuan untuk menyediakan mereka bagi mengambil peperiksaan Ijazah lanjutan. Latihan juga diberikan bagi meningkatkan pengetahuan dan kecekapan kakitangan sokongan. Aktiviti latihan yang diadakan termasuklah CME Jabatan , MO teaching sessions dan sebagainya.

Mesyuarat Jabatan

Mesyuarat Jabatan akan dijalankan sekurang-kurangnya sekali setiap 3 bulan bagi membincangkan sebarang masalah dan mencari jalan bagi meningkatkan produktiviti, kualiti, kecekapan dan keberkesanan. Mesyuarat akan dipengerusi oleh Ketua Jabatan dan dihadiri oleh anggota jabatan. Masalah yang tidak boleh diselesaikan diperingkat Jabatan akan dibawa ke Mesyuarat Pengurusan Hospital

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DASAR OPERASI WAD PSIKIATRI

ARAHAN KERJA JABATAN PSIKIATRI

Berikut adalah Arahan Kerja Jabatan Psikiatri dalam melaksanakan tugas rawatan pesakit psikiatri.

1. Arahan kerja kemasukan dan discaj pesakit
2. Arahan kerja ikatan fizikal pesakit
3. Arahan kerja pesakit lari dari wad
4. Arahan kerja pesakit suicidal
5. Arahan kerja kematian di wad
6. Arahan kes-kes ' medical condition with psychiatry manifestations'
7. Arahan kerja kes- kes yang dirujuk daripada jabatan kecemasan

1. ARAHAN KERJA KEMASUKAN DAN DISCAJ PESAKIT

Prosedur Kemasukan

1. Kemasukan sebagai pesakit dalam psikiatri di Hospital Serian hanya boleh dilakukan secara sukarela atau *voluntary*.
2. Pesakit boleh didiscaj setelah disahkan stabil oleh pegawai perubatan dari Jabatan Psikiatri dan Kesihatan Mental Hospital Serian setelah berbincang dengan Pakar Psikiatri bertugas. Jika pesakit didapati masih tidak lagi stabil dari segi kesihatan mental serta tidak sesuai untuk terus ditahan sebagai pesakit dalam psikiatri, keluarga mestilah dinasihati untuk membuat pemindahan ke Wad Psikiatri Hospital Sentosa ataupun Hospital Umum Kuching sebagai kemasukan tidak sukarela atau *involuntary* sebagaimana yang termaktub dalam Akta Kesihatan Mental 2001.
3. Kes- kes yang dinilai mempunyai risiko untuk bunuh diri yang sederhana atau tinggi adalah tidak sesuai dimasukkan sebagai pesakit dalam psikiatri di Hospital Serian. Ini kerana pemantauan rapi secara berkala (iaitu setiap 15- 30minit) adalah satu kemestian dan ianya tidak sesuai dilaksanakan di Hospital Serian.



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4. Bagi kes- kes daripada jabatan perubatan atau pembedahan tetapi pada masa yang sama mempunyai masalah kesihatan mental seperti kemurungan dan mempunyai risiko bunuh diri yang sederhana atau tinggi setelah dinilai oleh pegawai perubatan daripada Jabatan Psikiatri dan Kesihatan Mental Hospital Serian, kemasukan ke Wad Psikiatri Hospital Sentosa atau Hospital Umum Kuching hanya boleh dilakukan setelah keadaan perubatan atau pembedahannya disahkan stabil. Jabatan yang berkenaan turut dikehendaki merujuk kes tersebut ke jabatan yang sama di Hospital Umum Kuching jika dirasakan perlu.
5. Kes- kes yang boleh dimasukkan sebagai pesakit dalam psikiatri hanyalah kes- kes yang telah disahkan stabil dari segi perubatan (*medical/ surgical/ orthopaedic*).
6. Jika didapati bahawa pesakit perlu dimasukkan sebagai pesakit dalam psikiatri, maka salah seorang ahli keluarga (*next of kin*) atau penjaga perlulah sentiasa berada disamping pesakit sepanjang masa. Walaubagaimanapun, ianya masih tertakluk dengan perbincangan Pakar Psikiatri bertugas .
7. Penggunaan katil pesakit dalam psikiatri adalah melalui prosedur kemasukan biasa seperti pesakit- pesakit perubatan atau surgikal yang lain.
8. Penerimaan pesakit hanyalah melalui Jabatan Kecemasan dan Trauma Hospital Serian atau melalui perpindahan daripada wad- wad lain di dalam Hospital Serian.
9. Penggunaan katil pesakit dalam psikiatri hanya boleh dilakukan selepas pesakit diperiksa dan dibenarkan oleh Pakar atau Pegawai Perubatan Jabatan Psikiatri dan Kesihatan Mental. Penggunaan katil pesakit dalam psikiatri oleh jabatan lain hanya boleh diberikan dengan persetujuan dengan mana- mana Pakar atau Pegawai Perubatan Jabatan Psikiatri dan Kesihatan Mental Hospital Serian.
10. Pengurusan penggunaan katil pesakit dalam psikiatri termasuklah proses orientasi perlulah dilakukan dengan kerjasama staf- staf terbabit seperti Matron, Sister atau jururawat bertugas di wad masing- masing.
11. Semasa kemasukan bilik pesakit dalam psikiatri termasuklah tandas perlulah dipastikan berada dalam keadaan bersih dan selamat.
12. Pengambilan 'vital sign' dan "sleep behavioral charting" secara tetap juga perlulah dilakukan oleh staf- staf bertugas di wad terbabit.
13. Semasa kemasukan, pesakit perlulah diperiksa secara teliti bagi memastikan tiada peralatan tajam atau berbahaya seperti pisau, pisau cukur, jarum dan lain- lain.
14. Semua ubatan yang dibawa pesakit perlu disimpan di kaunter jururawat.

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15. Pesakit yang telah dimasukkan sebagai pesakit dalam psikiatri perlulah dilihat semula oleh pegawai perubatan psikiatri (pada hari dan waktu bekerja) atau oleh pegawai perubatan atas panggilan (di luar waktu bekerja) sekurang- kurangnya dalam tempoh dua jam dari masa kemasukan bagi kes- kes biasa ataupun dalam masa setengah jam jika diminta oleh staf yang bertugas.
16. Rondaan perlu dibuat sekurang- kurangnya sekali sehari oleh pegawai perubatan yang bertugas.
17. Jika pesakit perlu diikat atau *restraint*, kes mestilah dimaklumkan kepada pegawai perubatan terlebih dahulu. Setelah diikat, pesakit mestilah dinilai oleh pegawai perubatan dari Jabatan Psikiatri dan Kesihatan Mental Hospital Serian (pada hari dan waktu bekerja) atau oleh pegawai perubatan atas panggilan (di luar waktu bekerja) dalam masa tiga puluh minit panggilan dibuat
18. *Restraint chart* mestilah dilakukan dari tempoh pesakit diikat sehingga kepada pesakit dilepaskan dari segi ikatan. Walaubagaimanapun penilaian perlulah dibuat jika pesakit perlu dipindahkan ke wad psikiatri Hospital Sentosa atau Hospital Umum Sarawak.
19. Pakar psikiatri (jika ada) perlulah membuat rondaan sekurang- kurangnya sekali sepanjang kemasukan pesakit ke dalam wad.
20. Ringkasan discaj (discharge summary) perlulah dikeluarkan oleh pegawai perubatan bertugas di jabatan psikiatri pada hari pesakit didiscaj.
21. SOP ini adalah sah digunakan dari tarikh dokumen ini disemak sehingga adanya penambahbaikan yang perludilakukan dari semasa ke semasa.

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2. ARAHAN KERJA IKATAN FIZIKAL

Indikasi dan Tatacara Untuk Kekangan Fizikal (Restraint) atau Pengasingan

1. Kekangan fizikal atau pengasingan tidak boleh dilakukan ke atas kanak-kanak di bawah umur 12 tahun.
2. Hendaklah dipastikan keselamatan dan privasi pesakit dipenuhi sepanjang masa sewaktu pesakit menjalani kekangan fizikal atau pengasingan.
3. Pakar psikiatri perlu membuat penilaian ke atas pesakit yang telah menjalani kekangan fizikal melebihi 8 (lapan) jam untuk menentukan sama ada kekangan fizikal perlu diteruskan atau tidak.
4. Bagi pesakit yang menjalani pengasingan secara berterusan melebihi 8 jam, pesakit hendaklah dinilai semula oleh pakar psikaiatri yang tidak merawat (jika ada) untuk menentukan sama ada pengasingan perlu diteruskan atau tidak.
5. Bagi pesakit yang menjalani pengasingan secara berkala, sekiranya jumlah keseluruhan masa pengasingan melebihi 12 jam, pesakit hendaklah dinilai semula oleh pakar psikaiatri yang tidak merawat (jika ada) untuk menentukan sama ada pengasingan perlu diteruskan atau tidak.

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Tatacara Kekangan Fizikal

1. Bagi pesakit yang memerlukan kekangan fizikal, hendaklah ditempatkan di bahagian wad yang berhadapan dengan kaunter paramedik (acute bay)
2. Paramedik/ jururawat yang bertugas perlu menjalankan pemerhatian ke atas pesakit yang menjalani kekangan fizikal mengikut garis panduan yang telah ditetapkan seperti mana yang tertera dalam Akta Kesihatan Mental 2001.
3. Hendaklah dipastikan kawasan di mana pesakit menerima kekangan fizikal adalah bersih, cukup pengudaraan, pencahayaan yang baik dan suhu bersesuaian.
4. Kekangan fizikal hanya melibatkan kaki dan tangan pesakit.
5. Pesakit yang memerlukan kekangan fizikal yang berlarutan walaupun kekangan kimia telah diberikan secukupnya perlu dinilai semula oleh pakar psikiatri dan dipindahkan ke Hospital Sentosa atau Hospital Umum Kuching untuk rawatan selanjutnya kerana Hospital Serian tidak sesuai untuk perawatan pesakit sedemikian.

3. ARAHAN KERJA PESAKIT LARI DARI WAD

- i. Usaha mencari di sekitar wad hendaklah dijalankan sebaik sahaja pesakit lari dari wad disedari.
- ii. Pegawai perubatan tugas panggilan hendaklah dimaklumkan dengan segera.
- iii. Pegawai pakar tugas panggilan hendaklah dimaklumkan jika disyaki pesakit akan membahayakan orang lain atau dirinya.
- iv. Laporan polis dan "incident report" hendaklah dibuat setelah usaha mencari pesakit dijalankan di sekitar wad gagal.
- v. Jika boleh dihubungi melalui telefon keluarga hendaklah dimaklumkan dengan kadar segera. Jika tidak ada hubungan telefon, keluarga hendaklah dimaklumkan melalui balai polis terdekat.
- vi. Pegawai polis hendaklah dimaklumkan bahawa mereka mempunyai bidang kuasa untuk mengambil kembali pesakit untuk masuk semula ke wad adalah diperuntukan di bawah Mental Health Act 2001.

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4. ARAHAN KERJA PESAKIT SUICIDAL (MEMBUNUH DIRI)

- i. Pegawai perubatan hendaklah membuat penilaian kemungkinan pesakit mempunyai niat untuk membunuh diri bagi setiap pesakit. Jika risiko bunuh diri berada pada tahap sederhana atau teruk, pesakit perlulah dipindahkan ke Jabatan Psikiatri Hospital Umum Sarawak atau Hospital Sentosa sebagai rawatan dalam pesakit.
- ii. Apabila dikenal pasti sebagai berpotensi untuk membunuh diri hendaklah diberi rawatan dan pemerhatian yang intensif.
- iii. Pesakit yang dikenal pasti berpotensi membunuh diri hendaklah diberikan pemerhatian Suicidal caution dan dimaklumkan kepada pembantu perubatan bertugas.
- iv. Semua kakitangan setiap syif hendaklah menandatangani makluman suicidal caution sebelum memulakan tugas.
- v. Pemerhatian sekurang-kurangnya setiap 15 minit hendaklah dijalankan tanpa gagal.
- vi. Hanya pegawai perubatan atau pakar sahaja yang boleh membatalkan pemerhatian suicidal caution.
- vii. Seseorang yang diberi pemerhatian suicidal caution hendaklah diperiksa oleh pegawai perubatan sekurang-kurangnya sekali dalam sehari sekali.
- viii. Pemerhatian suicidal caution hendaklah ditempatkan di kawasan rawatan intensif.

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5. ARAHAN KERJA KEMATIAN PESAKIT DI WAD

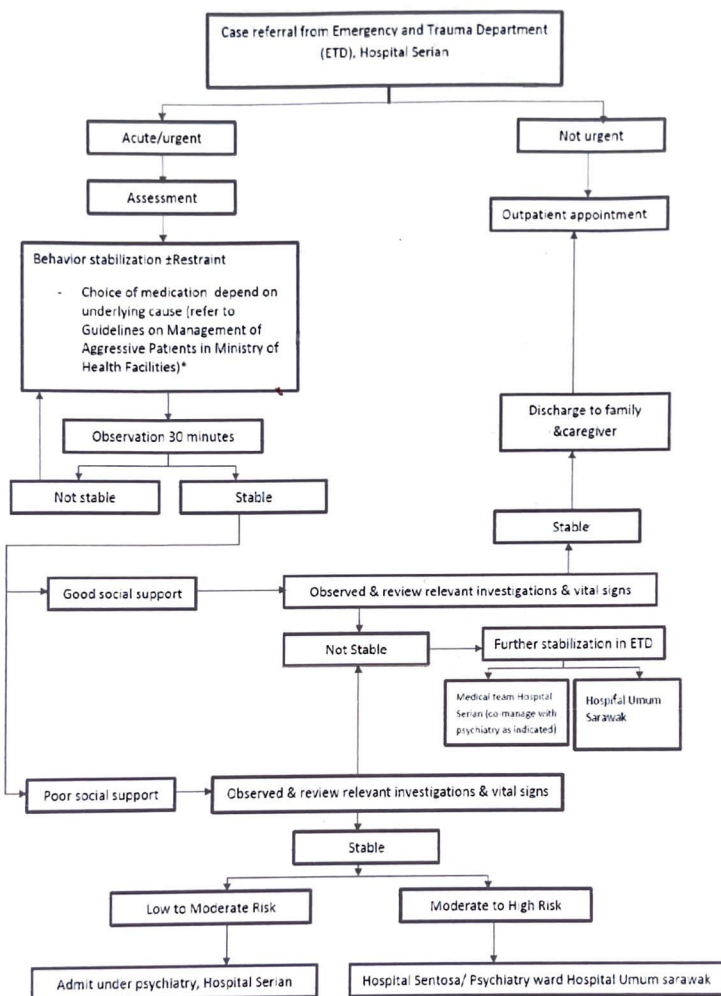
- Bila ada kematian mengejut dan punca kematian tidak diketahui di wad psikiatri, pegawai yang bertugas hendaklah memberitahu kepada Dr Oncall dan Pakar Oncall.
- Pegawai yang bertugas hendaklah menghubungi polis dan pegawai dari jabatan forensik untuk tindakan selanjutnya.
- Jika pihak keluarga yang terbabit tidak setuju dengan tatacara kematian mengejut di wad psikiatri iaitu postmortem, mereka perlu mendapat arahan dari pihak mahkamah.
- Pegawai perubatan yang bertugas dikehendaki mengisi sijil kematian selepas postmortem dibuat atau selepas mendapat nasihat dari pakar forensik.

6. ARAHAN KES KES ' MEDICAL CONDITION WITH PSYCHIATRY MANIFESTATIONS'

- Terdapat beberapa keadaan penyakit perubatan/surgical yang datang dengan manifestasi seperti penyakit psikiatri namun boleh membahayakan nyawa sekiranya tidak menerima pemerhatian dan rawatan perubatan/surgical yang sewajarnya. Oleh yang demikian, mana-mana kes yang mempunyai manifestasi penyakit psikiatri seperti psikosis yang akut, perubahan tingkah laku yang akut, gejala-gejala 'intoxication' atau 'withdrawal' daripada penyalahgunaan alkohol atau dadah seharusnya menerima pemerhatian pakar medikal/surgical dan menerima rawatan yang sewajarnya.
- Sekiranya pesakit memerlukan rawatan sokongan daripada pihak psikiatri semasa menerima rawatan medikal/surgical, pihak psikiatri akan bertindak sebagai 'liaison team' dan memberikan rawatan yang sewajarnya.
- Setelah penyakit perubatan/surgical telah dikecualikan dan keadaan fizikal pesakit stabil, pesakit akan diambil alih oleh pihak psikiatri untuk rawatan selanjutnya sekiranya diperlukan.

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7. ARAHAN KERJA KES KES YANG DIRUJUK OLEH JABATAN KECEMASAN



*To admit or observe in ETD if IM Midazolam was given until vital signs stable